

PARMOL

VIPAR10-0 (MY)

Descriptions

- Parmol 120 Syrup : Orange, viscous liquid with strawberry flavour.
- Parmol 250 Suspension : Pink, viscous suspension with raspberry flavour.

Compositions

- Parmol 120 Syrup : Paracetamol 120 mg/5 ml.
- Parmol 250 Suspension : Paracetamol 250 mg/5 ml.

Pharmacodynamics

Paracetamol has analgesic and antipyretic actions but no useful anti-inflammatory properties. It may act by inhibiting prostaglandin synthesis in the central nervous system (CNS) and through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of the synthesis of prostaglandins or to inhibition of the synthesis or actions of other substances, which sensitize pain receptors to mechanical or chemical stimulation.

Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

Pharmacokinetics

Absorption

Rapid and almost complete following oral administration; may be decreased if taken following a high-carbohydrate meal.

Blood Concentration

After an oral dose of 1g to fasting subjects, peak plasma concentration of 20 to 30 µg/ ml are attained in about 20 minutes. After giving the same dose to non-fasting subjects, peak plasma concentrations of 10 to 20 µg/ ml are attained in 40 to 120 minutes.

After an oral dose of 2g to fasting subjects, peak plasma concentrations of about 30 to 40 µg/ ml are attained in 1 to 3 hours.

Half-life

Plasma half-life after therapeutic doses is approximately 1 to 4 hours; does not change with renal failure but may be prolonged in some forms of hepatic disease, in the elderly, and in the neonate, and may be somewhat shortened in children.

Distribution

Widely distributing throughout most body fluids; secreted in the saliva in concentrations paralleling those in the plasma; volume of distribution, about 1 liter/ kg body weight.

Protein Binding

Plasma-protein binding is negligible with doses producing plasma concentrations below 60 mcg per ml but increases with increasing concentrations.

Metabolic Reactions

Approximately 90 to 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulfuric acid, and cysteine. The metabolite formed by oxidation, which is hepatotoxic, is conjugated by the glutathione pathway to form mercapturic acid and cysteine conjugates.

Excretion

About 85% of a therapeutic dose is excreted in the urine in 24 hours; of the excreted material in adult, 1 to 4% is unchanged, 20 to 30% is conjugated with sulphate, 40 to 60% is conjugated with glucuronic acid, 5 to 10% is composed of 3-hydroxy-3-sulphate, 3-methoxyglucuronide, and 3-methoxy-3-sulphate metabolites, and 5 to 10% is composed of mercapturic acid and cysteine conjugates. Larger amounts of the mercapturic acid and cysteine conjugates are excreted in overdose.

Overdose

Plasma concentrations have been used to indicate possible hepatic necrosis, and at 4 hours, hepatic necrosis is possible at concentrations of paracetamol above 120 to 300 µg/ ml. Similarly, at 12 hours, concentrations above 120 µg/ml indicate probable necrosis, concentrations of 50 to 120 µg/ml indicate that it is possible, and concentrations below 50 µg/ml indicate that it is unlikely. The hepatotoxic metabolic is normally detoxified by glutathione conjugation but once sources of glutathione are depleted the free metabolite is available to bind with liver cell protein. This binding occurs some 10 hours after dosing.

Indications

For relief of minor types of pain such as headache, myalgias, toothache and febrile conditions.

Contraindications

It is contraindicated in patients with known hypersensitivity to paracetamol.

Warnings and Precautions

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

Allergy alert:

Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Caution in patients with hepatic or renal function impairment. Check with physician if symptoms do not improve or if fever lasts longer than 3 days or recurs. Chronic use should be avoided, unless otherwise directed by physician.

Side/Adverse Effects

In recommended therapeutic dosage, paracetamol is usually well tolerated. Rarely, hypersensitivity and haematological reactions may occur.

Drug Interactions

Risk of hepatotoxicity with single toxic dose or prolonged use of paracetamol may be increased in chronic alcoholics or in patients regularly taking other hepatic enzyme-inducing agents.

Overdose

Clinical features:

Nausea, vomiting, epigastric discomfort, hypoprothrombinaemia, hepatic or renal failure and metabolic acidosis or coma in very severe cases.

Treat overdosage by emesis or gastric lavage, if indicated. Administer N-acetylcysteine orally (start within 10 hours after ingestion), 140 mg of 20% solution per kg as a loading dose, followed by 70 mg/kg every 4 hours for 3 days with the necessary supportive treatment if required.

Dosage and Administration

Parmol 120 Syrup

Children:

- 1 to 6 years : 1 to 2 teaspoonful (5-10 ml) 3 to 4 times daily.
- 7 to 12 years : 2 to 4 teaspoonful (10-20 ml) 3 to 4 times daily.

Parmol 250 Suspension

- Adult : 2 to 4 teaspoonful (10 to 20 ml) not exceeding 80 ml daily.

Children:

- 1 to 6 years : half to 1 teaspoonful (2.5 to 5 ml) 3 to 4 times daily.
- 7 to 12 years : 1 to 2 teaspoonful (5 to 10 ml) 3 to 4 times daily.

Note: The information given here is limited.

For further information, consult your doctor or pharmacist.

Storage: Store below 30°C. Protect from light and freezing.

Presentation/Packing:

- Syrup 120 mg/5 ml x 60 ml, 100 ml, 120 ml.
- Syrup 120 mg/5 ml x 3.6 L (For Export Only)
- Suspension 250mg/5 ml x 60ml, 120 ml.
- Suspension 250 mg/5ml x 3.6 L (For Export Only).

Product Registration Holder/
Manufactured by: HOVID Bhd.,
121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.